Histrionic Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

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1. What it is

Histrionic personality disorder (HPD) is a long-standing pattern of **excessive emotionality and attention-seeking**, beginning by early adulthood and present in many settings (American Psychiatric Association [APA], 2022). It belongs to **Cluster B** (dramatic or erratic).

How common: in a large U.S. national survey, 1.84% of adults met criteria (Grant et al., 2004). Across Western community studies, rates vary widely depending on assessment method (Volkert et al., 2018).

A diagnosis with a troubled history. HPD descends from the old concept of "hysteria," long tied to beliefs about women, religion, and health. It is considered one of psychiatry's most ambiguous categories (Novais et al., 2015). Clinicians should guard against gender stereotypes: men can meet criteria too, and lively, expressive personalities are not disorders.

Counselor framing: beneath the drama is usually a fragile sense of worth that depends on being noticed. The person often feels unseen, bored, and empty when attention fades.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive pattern, beginning by early adulthood, with **5 or more** of:

- **Uncomfortable when not the center of attention.**
- Interactions often marked by **inappropriately seductive or provocative** behavior.
- **Rapidly shifting and shallow** expression of emotions.
- Consistently uses **physical appearance** to draw attention.
- Speech that is **highly impressionistic and lacking in detail**.
- **Self-dramatization, theatricality,** and exaggerated emotional expression.
- **Suggestible,** easily influenced by others or circumstances.
- Considers relationships **more intimate than they actually are**.

Distinguish from: borderline personality disorder (self-harm, intense anger, emptiness, and identity disturbance are more prominent), narcissistic personality disorder (seeks admiration for superiority rather than any attention), dependent personality disorder, and mood episodes such as hypomania (APA, 2022).

When counselors must refer (general clinical consensus): refer for evaluation when there is depression, suicidal gestures or threats, substance use, or physical complaints that need medical review. Take every suicidal statement seriously, even if it seems attention-seeking.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in histrionic personality disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
SCID-5-PD (Structured Clinical Interview for DSM-5 Personality Disorders)	Diagnosing all ten DSM-5 personality disorders, with an optional self-report screener.	Semistructured clinician interview	Yes	★ Clinical interviewing skill; training recommended	APA Publishing

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PID-5 (Personality Inventory for DSM-5)	Dimensional personality trait profile (negative affect, detachment, antagonism, disinhibition, psychoticism); brief and informant forms.	Self-report; 220-item full or 25-item brief form	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
PAI (Personality Assessment Inventory)	Broad clinical and personality assessment for adults, including scales for borderline and antisocial features.	Self-report, 344 items, about 50 to 60 minutes	Yes	★ Level C	PAR
MCMI-IV (Millon Clinical Multiaxial Inventory-IV)	Adults in treatment; personality patterns alongside clinical syndromes.	Self-report, 195 true/false items, about 25 to 30 minutes	Yes	★ Level C	Pearson
MMPI-3 (Minnesota Multiphasic Personality Inventory-3)	Broad personality and psychopathology assessment, with validity scales for over- and under-reporting.	Self-report, 335 true/false items, about 25 to 50 minutes	Yes	★ Level C	Pearson
PHQ-9 (Patient Health Questionnaire-9)	Screening for co-occurring depression.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: research on HPD is very thin. Schema therapy has trial support across a group of personality disorders that included HPD; otherwise, treatment relies on cognitive and interpersonal methods.

Evidence overview (honest effect sizes):

- **Schema therapy:** in a multicenter trial of 323 patients with avoidant, dependent, obsessive-compulsive, paranoid, histrionic, or narcissistic personality disorder, schema therapy (which targets long-standing core beliefs about self and others) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014). Few participants had HPD, so HPD-specific conclusions are uncertain.
- **No HPD-specific trials:** we found no randomized trial of any psychotherapy or medication for HPD alone (literature search, November 1, 2026).
- **Medication (referral notes):** no medication treats HPD itself. Co-occurring depression or anxiety may be treated in the usual ways.

Cognitive work with HPD: how it is used (Bamelis et al., 2014):

- **Keep sessions focused.** Gently ask for specifics ("What exactly happened next?") to counter vague, impressionistic speech.
- **Identify and challenge core beliefs:** "I am nothing unless people notice me," "If I'm not charming, I'll be ignored," "My feelings must be shown dramatically to be taken seriously," "This person really loves me" (after one meeting).
- **Build inner worth and real intimacy:** practice tolerating ordinary attention, expressing feelings directly and calmly, and building relationships slowly.

5. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion or spirituality in histrionic personality disorder** and no faith-integrated treatment trial (literature search, November 1, 2026).

Related evidence. Across 49 studies, positive religious coping went with better adjustment to stress and negative religious coping with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

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1. Schema-focused and cognitive approaches are the best available guide. Faith integration supports them; it does not replace them.
2. There is no research on faith and HPD. Church settings can either feed the need for attention (performance, public emotion) or offer steady acceptance that does not depend on performance.
3. Avoid labeling expressive worship or emotion as pathology; the concern is a pervasive, impairing pattern.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within cognitive, schema-focused care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for worth tied to being seen, including in church: "If I'm not up front, I don't matter," "God only notices me when I'm on fire for Him," "People will only love me if I'm exciting." To identify deeper false beliefs, see: Core Beliefs, Parental Messages, or use the Semantic Search Tool.
- **Challenge (clinical).** Examine the evidence for "I am nothing without attention," and test what happens when the client serves quietly or expresses feelings calmly (Bamelis et al., 2014). **Possible cognitive errors** to watch for, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
  - **All-or-nothing thinking** — no middle ground. *Feeds:* attention-seeking. *Sounds like:* "If I'm not the center of attention, I'm invisible."
  - **Jumping to conclusions** — deciding without the facts. *Feeds:* seeing relationships as closer than they are. *Sounds like:* "We just met, but she's my best friend."
  - **Emotional reasoning** — I feel it, so it must be true. *Feeds:* rapidly shifting emotions. *Sounds like:* "I feel it so strongly, it must be true."
  - **Mind reading** — assuming I know what others think. *Feeds:* discomfort when not noticed. *Sounds like:* "They're bored with me."
  - **Catastrophizing** — expecting the worst possible outcome. *Feeds:* dramatic expression. *Sounds like:* "This is the worst thing that has ever happened."
  - **Comparisons** — measuring myself against others. *Feeds:* envy. *Sounds like:* "She gets all the attention at church."
- **Questions to dispute the belief.** Have the client choose questions from the Questioning False Beliefs form that "speak to" the belief. To see how, watch Dr. Campbell's video Integrated Christian Counseling: Questioning Your Beliefs. Selected questions that fit the errors above:
  - "How might an all-loving God look at me or this situation?" and "Does everyone have to like me in order for me to be happy?" *For:* all-or-nothing thinking and worth tied to attention.
  - "Are there other ways of looking at this situation?" *For:* jumping to conclusions.
  - "What's the actual evidence for my belief?" *For:* emotional reasoning.
  - "Is it possible that I am distorting the facts?" *For:* mind reading.
  - "Am I blowing things up way out of proportion?" *For:* catastrophizing.
  - "Is this really the most important thing in life?" *For:* comparisons.
- **Replace (pastoral).** Offer Scripture that roots worth in God's unchanging gaze, not in an audience:
  - "…the LORD does not see as man does. For man sees the outward appearance, but the LORD looks at the heart." (1 Samuel 16:7)
  - "Am I now seeking the approval of men, or of God? Or am I striving to please men? If I were still pleasing men, I would not be a servant of Christ." (Galatians 1:10)
  - "I praise You, for I am fearfully and wonderfully made. Marvelous are Your works, and I know this very well." (Psalm 139:14). Worth that is given, not earned by performance.
  - "The LORD your God is among you; He is mighty to save. He will rejoice over you with gladness…" (Zephaniah 3:17). Already delighted in.
- **Faith community (clinical).** Offer warm, steady relationships with clear limits. Give service roles that build character and connection rather than constant spotlight.
- **Ministers:** respond calmly and consistently, keep boundaries clear (including around flirtation or overstated closeness), and refer when there is depression or any talk of self-harm.

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7. Quick classroom takeaways

- HPD is pervasive attention-seeking and shallow, shifting emotion (5 of 8 criteria). About 1.8% of U.S. adults; beware gender bias.
- Evidence is thin; schema therapy helped a mixed group of personality disorders that included HPD.
- Replace "I am nothing unless noticed" with the truth that God sees the heart and already delights in His children.
- Confirm with a structured interview (SCID-5-PD) rather than impressions alone, which helps guard against gender bias.

Sources note

All 7 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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